

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act

Part One — What Real Accessibility and Inclusion Actually Look Like

Section 1.2 — The Lifecycle Frame: Meeting People at Their Level

Section 1.1 established the definitions: accessibility is the floor, inclusion is the standard, and both are binding obligations under federal statute and international treaty. Section 1.2 establishes the principle that governs how those obligations apply across a human life. The principle is straightforward and the system Alberta has built does not honour it. *A fair province meets disabled people at their level, at every life stage, with the kind of support their condition actually requires* — not the kind of support a politically convenient template imposes.

Disability is not a single condition or a single life stage. It enters lives at birth, in childhood, in adolescence, in working age, in middle age, in late life. It enters through genetics, through accident, through illness, through age, through chance. It produces fixed conditions, progressive conditions, fluctuating conditions, and conditions that change in unpredictable directions over decades. A support architecture designed for one disability category, one age, or one trajectory will fail every category, age, and trajectory it was not designed for. The lifecycle frame is the recognition of that reality and the structural response to it.

The Three Modes of Support

Disability support, properly designed, operates in three distinct modes. Each mode is appropriate to a different population and a different life stage. A fair system identifies which mode applies to each person and provides the architecture that mode requires. The modes are not hierarchical. None is superior to any other. They are different responses to different clinical realities, and the absence of any one of them produces predictable harm to the populations who needed it.

Growth

The growth mode applies where intervention can produce capacity development. Three populations belong here. Children with developmental disabilities are in the growth mode during the developmental windows of early childhood, when the brain's neuroplasticity makes intervention not merely beneficial but irreplaceable. Adults recovering from acquired disability — brain injury, stroke, spinal cord injury, post-surgical limitation — are in the growth mode during rehabilitation, when targeted therapy can restore or rebuild function. Adults adapting to newly diagnosed conditions are in the growth mode during the period of learning accommodations, assistive technology, and adapted skills.

The growth mode requires investment. Therapy. Specialist access. Equipment. Education. Vocational training where employment is realistic. The investment has documented returns: every dollar invested in early childhood developmental intervention produces multiple dollars in averted later costs, according to research summarized by the Public Health Agency of Canada and consistent across the international

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literature on early intervention efficacy. The growth mode is the most cost-effective mode the disability support system contains. A province that under-invests in growth-mode supports pays the cost downstream, in higher needs across the other two modes for decades afterward.

Alberta currently under-funds the growth mode catastrophically. The Family Support for Children with Disabilities (FSCD) program, the principal provincial vehicle for childhood disability support, operates with wait times that approach three years in some regions. Section 1.3 of this document examines what is foreclosed in that wait. The clinical answer is unambiguous: developmental windows close, and the capacity that intervention would have produced is not recoverable later. The fiscal answer is that the province pays for that foreclosure for the rest of those children's lives, through every subsequent program that has to absorb needs the growth mode would have prevented.

Stability

The stability mode applies where the condition is managed, not cured, and where the appropriate architecture is consistent baseline support across time. Adults with chronic illness, fluctuating conditions, mental health conditions in maintenance, autoimmune disease, post-rehabilitation acquired disability, and most adult-onset disabilities live in the stability mode. The defining feature of these populations is not that they cannot work. It is that their capacity is variable, their needs are continuous, and the supports that allow them to function are the supports that must not be disrupted.

The stability mode requires the opposite of the testing-and-reassessment culture Alberta has built. It requires predictability. Consistent baseline income. Reliable medical coverage. Low-friction access to the supports that keep the condition managed. Continuity of caseworker, healthcare provider, and program rules. A stability-mode population cannot recover from constant administrative disruption, because the disruption itself becomes the destabilizing force. Every reassessment burden, every form to refile, every appeal to mount, every pharmacy denial to contest, consumes the energy the population has available for functioning. The cost is not theoretical. It is the documented pattern of mental health collapse, housing instability, and secondary disability that the AISH population describes constantly.

Alberta's current AISH-to-ADAP transition is a stability-mode failure. The transition imposes mass reassessment on a population whose condition is not changing — only the program testing them is. The 79,290 Albertans currently on AISH are, overwhelmingly, a stability-mode population. The government's own approval processes already established that. The reversal of that approval, en masse, without new clinical evidence and without independent appeal, is the imposition of growth-mode testing logic on a stability-mode population that does not benefit from it and is harmed by it.

Lifelong Support

The lifelong support mode applies where the condition requires ongoing direct intervention with no expectation of independent self-management. Profound intellectual disability. Severe autism with high support needs. Advanced ME/CFS. Severe psychiatric conditions in active phase. Late-stage progressive neurodegenerative disease. Severe acquired brain injury with permanent care needs. Aging-related cognitive decline at advanced stages. These populations require permanent support architecture — supervised housing, group homes, day programs, family caregiver support that compensates families for the work the system would otherwise do, continuing care facilities, and the income supports that make all

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of the above sustainable.

The lifelong support mode is the mode the province most aggressively under-builds. The Persons with Developmental Disabilities (PDD) program is the principal provincial vehicle for adults requiring lifelong support, and PDD waitlists have not meaningfully moved in years. The Office of the Alberta Ombudsman's 2024 report *Denied by Design* documented systematic failures in PDD intake, eligibility, and service delivery. Adults with profound intellectual disability live with aging parents into those parents' deaths because no PDD-funded placement is available. The cost is borne by families until the families collapse. Then the cost is borne by emergency systems — hospitals, justice, homelessness response — at multiples of what proper lifelong-support architecture would have cost.

The lifelong support mode is also the mode most vulnerable to the Growth-mode testing logic that the AISH-to-ADAP transition embodies. Asking an adult with profound intellectual disability to demonstrate employment readiness, complete a 27-page reassessment, or attend a phone-tree appointment is not reform. It is structural cruelty applied to a population that the system already determined required permanent support. There is no growth here to test for. The test is the harm.

THE THREE MODES OF DISABILITY SUPPORT

Growth — for populations where intervention produces capacity development. Children in developmental windows. Adults in rehabilitation after acquired disability. Adults learning accommodations for newly diagnosed conditions. Requires therapy, specialist access, education, equipment, and vocational support where realistic. Highest cost-benefit ratio in the system.

Stability — for populations whose conditions are managed, not cured. Chronic illness, fluctuating conditions, mental health in maintenance, autoimmune disease, post-rehabilitation acquired disability. Requires predictability, consistent baseline income, low-friction access, and the absence of adversarial reassessment culture. Disruption itself produces secondary harm.

Lifelong Support — for populations requiring ongoing direct intervention with no expectation of independent self-management. Profound intellectual disability. Severe autism with high support needs. Advanced progressive disease. Requires supervised housing, day programs, family caregiver compensation, continuing care, and the income supports that sustain all of the above. Failure here produces family collapse and emergency-system displacement.

The principle: a fair province identifies which mode applies to each person and provides the architecture that mode requires. Substituting one mode for another — particularly substituting growth-mode testing for stability or lifelong-support populations — produces predictable, documented harm. Alberta is currently making that substitution at scale.

Why Single-Mode Systems Fail

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The defining error in Alberta's current direction is the application of a single mode — growth-mode logic, expressed as employment-readiness, work expectations, and reassessment culture — to populations whose actual needs are stability or lifelong support. The error is structural, not individual. It cannot be fixed by softer rhetoric or better caseworker training. It can only be fixed by building a support architecture that recognizes the three modes as distinct and provides each mode in its appropriate form.

A growth-only system fails the stability population by destabilizing them through testing they cannot pass and do not need. It fails the lifelong-support population by demanding capacity development from people whose conditions preclude it. The harm is documented across the disability literature internationally. Every jurisdiction that has tried to fold permanent disability into employment-readiness frameworks has produced the same outcomes: population destabilization, increased downstream costs, family collapse, and measurable mortality.

A stability-only system fails the growth population by under-investing in the developmental windows where investment produces returns. It fails the lifelong-support population by treating them as if maintenance is sufficient when active support is required. The result is what Alberta's PDD population currently faces: no growth-mode investment in childhood, then static maintenance through adolescence, then nothing at age 18 when the school system ends, then a waitlist for the lifelong-support architecture that should have caught them at the threshold.

A lifelong-support-only system fails the growth population by under-investing in their developmental potential. It fails the stability population by treating manageable conditions as if they require institutionalization. No jurisdiction operates this way; the example is included only to make the symmetry of the failure visible. The point is that all three modes are required, in their appropriate proportions, applied to the populations they were designed for. Substitution does not work in any direction.

What This Frame Requires of Alberta

The remainder of Part One examines Alberta's performance against the lifecycle frame at each stage. Section 1.3 examines the growth mode at its most consequential life stage — early childhood, where the FSCD program operates and where developmental windows close on a documented timeline that the wait list ignores. Section 1.4 examines the transition from childhood supports to adult supports at age 18, where the growth, stability, and lifelong-support modes must be properly assigned, and where Alberta currently strands large portions of the population by failing to build the bridge. Section 1.5 examines the role of family caregivers, who currently absorb the cost of every mode the province under-funds, and what happens when the primary caregiver dies. Section 1.6 examines acquired disability — the moment a previously non-disabled Albertan enters the system — and the comparator question that closes Part One: why the province responds with full triage architecture to acquired disability through workplace injury (the WCB model), and refuses comparable architecture to disability that arrives through any other route.

The blueprint Part Three builds is the architecture that delivers all three modes, properly assigned, across the lifecycle. The legislation Part Three proposes is the binding mechanism that protects that architecture from the political volatility that has produced the current dismantling. The argument Part Four makes is that the dismantling is a choice, made deliberately, by ministers with full biographical knowledge of what their decisions cost. The closing argument Part Five makes is that what is being dismantled is the

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architecture every Albertan will eventually need, in some mode, at some life stage, through some route none of us can predict.

The lifecycle frame is the foundation that argument is built on. A province that does not meet disabled people at their level, at every stage, with the support their condition actually requires, has not built accessibility and inclusion. It has built a system that processes disabled people through templates designed for someone else, and discards the populations the templates do not fit. That is the system Alberta currently has. The blueprint is the architecture that replaces it.